

REVIEWS



A Review of Well-being Assessment Instruments in Hospital Medicine

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ABSTRACT

Although many well-being measurement instruments are used by hospitalist groups, the benefit of one instrument over others has not been established. We performed a review of well-being measurement instruments used by hospital medicine groups and developed a framework for hospitalists to choose an instrument to best evaluate their group's well-being. The review was performed using the following databases: PubMed, Ovid MEDLINE, Embase, Cochrane, CINAHL, Psych INFO, Web of Science, and Scopus. After deleting repeat citations, a total of 330 citations were identified. Two authors read the titles, abstracts, and papers as necessary to identify papers that measured hospitalist well-being. The search identified 35 papers that used well-being instruments to identify 11 specific instruments used. Once instruments were identified, the author group reviewed the papers to determine which instrument features were important for inclusion in a summary table. The authors used the drivers of burnout as identified by Shanafelt et al. to evaluate the benefit of the instruments. Of the 11 most relevant well-being measurement instruments identified, the majority assess three or fewer categories of drivers of burnout as identified by Shanafelt et al. with only three instruments measuring five or more drivers. The review provides a practical guide for hospital medicine groups to select the most suitable instrument depending on their unique needs, including the need for a general overview of hospitalist well-being, the need to identify specific drivers, or the need to evaluate the impact of an intervention.

KEY WORDS: well-being; Hospitalists; burnout

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CASE EXAMPLE

A hospital medicine group of about 50 physician hospitalists is facing the sudden departure of three of its long-term well-respected hospitalists. Each departing hospitalist is staying in the general area and has taken on a new role at one of several competing hospitals. In addition, fewer hospitalists have been interested in picking up extra shifts for additional pay, which the group relies upon for staffing needs. Leadership is concerned that burnout may be the source of these changes and wants to study the level of burnout in the group and what factors may be driving it. However, the team is uncertain about how to assess well-being and burnout within their team. Specifically, they are unfamiliar with existing wellness measurement instruments. They are interested in identifying an instrument that is relevant to hospitalists, answers the questions they have about their group's wellness, and gives them ideas for interventions. This paper directly addresses their problem by offering a review of well-being instruments commonly used by hospital medicine groups, analyzing their relevance for hospitalists, mapping each instrument to essential drivers of burnout and providing recommendations for instrument selection in various circumstances. By distilling complex literature into a practical guide designed specifically for hospitalists, the paper empowers leadership of hospital medicine groups to make informed decisions about selecting the most suitable instrument to measure, understand, and ultimately address burnout among their clinicians.

INTRODUCTION

Physician burnout, which can be defined as poor well-being, has gained increased attention in recent years and high rates of workplace dissatisfaction have provided a compelling impetus for driving change within medicine.¹ Recent work in this area identifies many of the root causes of burnout and highlights the impact of healthcare worker well-being on healthcare quality and patient safety.² Levels of burnout for hospitalist physicians remain near all-time highs with estimates of 40–60% of hospitalists experiencing

burnout,^{3,4} compared to 33% in 2015.⁵ These trends not only impact patient safety but also have a profound impact on the workforce, correlating with physician turnover, reductions in clinical time, and physicians leaving hospital medicine altogether.^{2,6}

Though healthcare institutions recognize the importance of addressing physician well-being, there is no standardized best practice for how hospital medicine leaders measure burnout and well-being within their groups. Many instruments are currently being used to measure drivers related to well-being. Each of these instruments uniquely defines wellness (resilience, burnout, quality of life, etc.) and explores the drivers of burnout to a variable extent. Each instrument also has variable relevance for hospitalists. Many are not validated or studied specifically in the hospitalist population. The drivers of burnout for hospitalists may be significantly different than for clinicians in other practice environments, given the stressors of inpatient medicine, particularities of hospitalist schedules, and the kind of patient mix and work burden hospitalists encounter.⁷

In this paper, we present a review of the most common well-being measurement instruments used for hospitalists based on a literature search. We analyze the instruments and assess various characteristics, including their relevance for hospitalist clinicians. Importantly, we explore how well each instrument captures systematic drivers of well-being and categorize these into seven domains of burnout drivers as defined by Shanafelt et al.⁸: workload and job demands, control and flexibility, work-life integration, social support and community at work, organizational culture and values, efficiency and resources, and meaning at work. We will henceforth refer to these domains as the “drivers.” The result is a comprehensive review of well-being measurement instruments as they apply to hospital medicine distilled into a practical and concise guide to help hospital medicine groups select the instrument best suited for the needs of their program.

METHODS

Workgroup Characteristics

The authors of this paper belong to the Hospital Medicine Reengineering Network (HOMERuN) Wellness Workgroup. This group was formed in 2020 to identify barriers to hospitalist well-being as a first step towards developing interventions. It consists of academic hospitalist physicians across the USA who have expertise in hospitalist well-being. Many members hold leadership roles related to well-being within their institutions and collectively have over 30 years of experience, numerous peer-reviewed publications, and national presentations in this area.

Data Sources and Searches

We performed a search for papers that evaluated hospitalist well-being to identify instruments for inclusion. The following databases were included in the search: PubMed, Ovid MEDLINE, Embase, Cochrane, CINAHL, Psych INFO, Web of Science, and Scopus. All authors proposed search terms independently. The author group collectively determined the final list of terms: hospitalists, wellness, well-being, morale, burnout, engagement, resilience, moral injury, moral distress, mental health, mental stress, work engagement, psychological resilience, distress syndrome, depression, cross-sectional study, prospective study, and longitudinal study with associated words also included. The full list of synonyms is included as Appendix 1. This initial search was completed with the assistance of a medical librarian on August 10, 2023. All peer-reviewed papers, including abstracts, published prior to the date of the search were included in the initial dataset. Studies not in English were reviewed with the assistance of a translation software. Duplicate studies were deleted from the results by the librarian initially and then one author (AN).

Study Selection

Two members of the workgroup (AN, KK) read the titles, abstracts, and papers as necessary to identify papers that named instruments used to evaluate hospitalist well-being. A third reviewer (EM) resolved discrepancies between the initial two reviewers.

From this list of papers, papers went through a second cycle of review. Papers that did not evaluate hospitalist well-being were excluded. Also, instruments that were more often used to screen for health instead of well-being, as determined by group consensus, were excluded. Instruments no longer available for widespread use, unable to be accessed via publications, or created solely for internal use within an institution were also excluded.

Data Extraction

As no standard rubric or appraisal instruments were available for use, the authors used a standard approach for data extraction with the goal of minimizing bias. Once instruments were identified, the authors evaluated the instruments using a structured evaluation protocol as follows: The column headings and details for each instrument were determined based on group consensus over the course of multiple iterative cycles. Each instrument was reviewed initially by at least two authors independently and then presented to the full workgroup at least four times. The initial discussion of each instrument included reviewing the instrument’s characteristics including length of instrument, what is specifically being measured by the instrument (what measure of well-being), and whether the instrument has costs associated

with its use. Data from the original publication about the instrument was included when possible. An additional search was completed to find publications that examined the instrument's validity.

As this is a review, IRB review was not required. No authors received grant funding to complete this review.

RESULTS

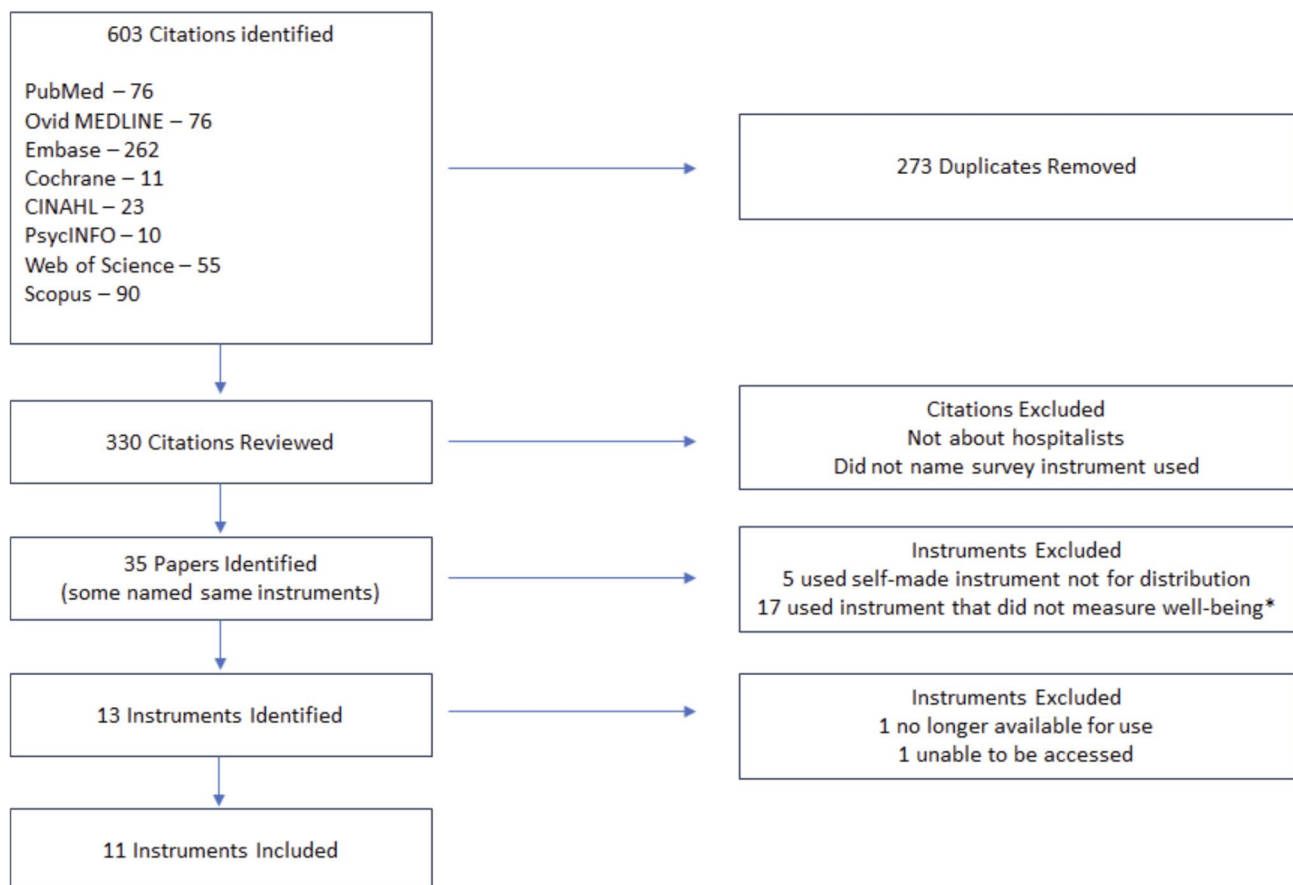
The initial search resulted in 603 records. After the removal of duplicates, 330 papers and abstracts remained for review. After AN and KK reviewed these citations with EM serving as a third reviewer for discrepancies, 35 papers were identified that evaluated hospitalist well-being. Appendix 2 lists all papers and the instruments identified from these papers. These papers were reviewed again by the full workgroup. Five instruments were excluded as they were created for internal use only. Seventeen instruments were excluded by group consensus as being unrelated to hospitalist well-being (most only focused on health).

One instrument was excluded due to lack of availability: The Hospital Physician Work-Life Survey was created by the Society of Hospital Medicine Job Satisfaction Task

Force for use in a national survey and is no longer available. An additional instrument, the Arbeitsbezogenes Verhaltens- Und Erlebensmuster (AVEM), was excluded as the authors were unable to access any publications that listed all questions within the instrument despite a robust search. This information is presented in Fig. 1.

The workgroup selected 11 instruments for review and compiled pertinent information for each including length and cost, validated population, and additional key features determined by the authors' expert opinions and consensus. The group paid particular attention to whether each instrument evaluated drivers of well-being, clustering the drivers assessed in each instrument into Shanafelt's framework of burnout drivers.⁸ This information is presented in Table 1.

The table's first five components were derived from the instruments and their associated resources: what the instrument measures as defined by the instrument (e.g. burnout, moral distress, or professional fulfillment), length, cost, validated population, and drivers of burnout assessed by the instrument. The authors chose to summarize and cluster the drivers assessed by each instrument into seven main categories discussed above in the Shanafelt and Noseworthy



* As defined by terms in Appendix 1 and group consensus

Figure 1 Study selection flowchart to identify well-being measurement tools for hospitalists.

Table 1 Table of Instruments

Name of instrument	What is being measured (as defined by the instrument)	Length	Cost	Validated population	Categories of driver dimensions assessed*	What is the benefit of this instrument	Link to instrument and/or reference**
Maslach Burnout Inventory-Human Services Survey (MBI-HSS)	Burnout as assessed by 3 categories: emotional exhaustion, depersonalization, and personal accomplishment	22 Likert-type questions	Cost differs depending on survey administration format and data access	Multiple populations including physicians (not specific to hospitalists)	Workload and job demands, meaning in work	Most commonly adopted tool for evaluation of burnout in health professions in published studies. National benchmark data available	https://www.mindgarden.com/314-mbi-human-services-survey
Maslach Burnout Inventory-2 (MBI-2)	Burnout as assessed by 2 categories: emotional exhaustion and depersonalization	2 Likert-type questions	Free	Not validated, correlates with full MBI-HHS tool†	None	The instrument can be completed quickly	Li-Sauerwine 2020 ⁹
Measure of Moral Distress for Health Care Professionals (MMD-HP)	Moral distress	27 two-part questions each with Likert scale, two optional write-in 2-part questions with Likert scale, two additional multiple choice questions	Free	Medical personnel including physicians (not specific to hospitalists)	Workload and job demands, control and flexibility, social support and community at work, organizational culture and values, efficiency, and resources	Made specifically for medical professionals	Epstein 2019 ¹⁰ supplement Table 1 1
Patient-Reported Outcomes Measurement Information System (PROMIS)	Physical, mental, and social health	Customizable subsets of > 1900 questions	Free‡	Some measures validated, not with medical personnel	Work-life integration	Measures the physical, social, and mental health of the group. Customizable	https://www.healthmeasures.net/explore-measurement-systems/promis
Mini-Z-version 2	Burnout, satisfaction, and stress	10 Likert scale plus 1 open-ended question	Free§	Medical personnel including physicians and APPs (not specific to hospitalists)	Workload and job demands, control and flexibility, work-life integration, social support and community and work, organizational culture and values, efficiency, and resources	Made specifically for physicians. Assesses many categories of drivers. Write-in qualitative question allows for identification of group-specific drivers. A “healthy” score for a workplace is defined	https://www.professionalworklife.com/mini-z-version
Hospitalist Morale Index (HMI)	Morale as assessed through burnout, emotional exhaustion, depersonalization, callousness	27 Likert scale questions	Contact author for information	Hospitalist physicians and APPs	Workload and job demands, work-life integration, social support and community at work, organizational culture and values, efficiency and resources, meaning in work	Made specifically for hospitalists. Assesses many categories of drivers. Tracks intent to leave	Chandra 2016 ¹¹ supporting information, appendix A
Professional Fulfillment Index (PFI) and Self-Valuation Measures	Professional fulfillment, burnout, and self-valuation/self-compassion	20 Likert scale questions	Free¶	Physicians (not specific to hospitalists)	Control and flexibility, work-life integration, social support and community at work, meaning in work	Made specifically for physicians. Accessible for individual use, in addition to group use. Provides real-time feedback and recommendations based on results. National benchmark data available	https://wellmd.stanford.edu/self-assessment.html

Table 1 (continued)

Name of instrument	What is being measured (as defined by the instrument)	Length	Cost	Validated population	Categories of driver dimensions assessed*	What is the benefit of this instrument	Link to instrument and/or reference**
Well Being Index-Expanded 9-item Well-Being Index (eWBI)	Fatigue, depression, burnout, anxiety/stress, and mental/physical quality of life	9 questions total, 7 yes/no questions plus 2 Likert scale questions	Free [#]	Medical personnel including physicians and APPs (not specific to hospitalists)	Workload and job demands, work-life integration, meaning in work	Made specifically for physicians. Quick status check of group wellbeing. Can compare to national data if paid subscription bought. Platform provides a space to track individual or group data over time	https://app.mywellbeingindex.org/assess
Professional Quality of Life (ProQOL) Health	Compassion satisfaction, burnout, secondary trauma stress	30 Likert scale questions	Free	ProQOL validated in multiple populations including physicians. ProQOL Health not validated ^{¶¶} . Neither specific to hospitalists	Workload and job demands, work-life integration, social support and community at work, meaning in work	Quick status check of group well-being. Nuanced questions can identify individual sources of burnout. Additional feedback and recommendations available based on results. Results can be stratified as low, average, and high based on ProQOL data	https://proqol.org/proqol-health-1
Copenhagen Burnout Inventory (CBI)	Personal burnout, work-related burnout, and client ^{††} (patient)-related burnout	19 Likert scale questions	Free	Multiple populations including physicians (not specific to hospitalists)	Workload and job demands, work-life integration, meaning in work	Quick status check of group well-being to assess if subcategories (personal vs work vs client) are driving burnout	Kristensen 2005 ¹² Table 2 2
Oldenburg Burnout Inventory (OLBI)	Exhaustion and disengagement	16 Likert scale questions	Free	Multiple populations including physicians (not specific to hospitalists)	Workload and job demands, work-life integration, meaning at work	Quick status check of group well-being. Disengagement questions can help track those at risk of quitting	Demerouti 2010 ¹³ appendix

*As previously discussed, categories from Shanefelt⁸

**Direct survey links included if available. If the instrument is located within a cited article, a citation is provided

[†]Score of greater than 3 correlates with burnout with a sensitivity of 93.6% and specificity of 73% compared to the full MBI-HHS⁹

[‡]For paper administration and scoring to group. Cost per measure for platform administration and scoring

[§]For use in research and education. Contact for commercial use

[¶]For non-profits for research or program evaluation. Must contact for permission to use

[#]For use for non-commercial. Pay version with enhancements including data analysis and resources related to results

^{¶¶}Validation studies in process

^{††}“Clients” is a broad concept covering terms patients, students, etc. When the CBI is used in practice, the term appropriate for the specific group of respondents is used

article.⁸ The categories in the Shanafelt and Noseworthy paper are concise, comprehensive, aptly labeled, and easy to comprehend, and most importantly, can point hospital medicine leaders in specific directions regarding well-being interventions and goals.

For the final component of the table, the authors described the benefits of each instrument based on the previous components and their expert opinion. The authors recognize that this is a fundamentally subjective assessment but utilized a rigorous process to reach consensus. Instruments were reviewed in-depth by multiple authors several times, discussed, and re-discussed among the author group over at least six meetings over the time course of a year, and on occasion, clarifications were obtained from the instrument creators.

The instruments varied in length, from the shortest, Maslach Burnout Inventory-2 (MBI-2) being two Likert scale questions to Patient-Reported Outcomes Measurement Information System (PROMIS), which can be customized with over 1900 questions. Most of the instruments are free for use for internal use for a group. Most are validated with healthcare professionals, but only the Hospitalist Morale Index (HMI) is specifically validated with a hospitalist population. The number of drivers addressed by the instruments varied, with PROMIS only addressing one driver and the HMI and Mini-Z version 2 addressing the most, six drivers.

DISCUSSION

The authors conducted a comprehensive literature review to identify and extensively review 11 instruments most used by hospital medicine groups. The authors then examined and discussed these instruments with a focus on relevance to hospitalists, based on their experience as hospital medicine and institutional well-being leaders. Hospitalists' work environments and drivers of burnout are complex and different enough from other fields to warrant evaluating well-being issues with instruments relevant to their practice environment.

The authors evaluated the utility of a specific instrument by how actionable the instrument's data was and in what way it can be used. Can the instrument lead to a relevant change that can feasibly be implemented by a hospital medicine group? With this as the foundation, it became clear that which instrument is best for measuring well-being is most dependent on why a leader or group is measuring well-being. There may be occasions when hospital medicine groups are interested in a quick measurement of general well-being. Indications for a general measurement could include a needs assessment to justify the creation of a workgroup or a check to see if a specific intervention led to improvements. In these circumstances, most of the instruments will provide a basic well-being measurement. The MBI-2, Well-Being Index

(eWBI), Professional Quality of Life (ProQOL) Health, Oldenburg Burnout Inventory (OLBI), Mini-Z, and Copenhagen Burnout Inventory (CBI) may be the most specific for this use.

At other times, hospitalist groups need well-being measurement instruments that comprehensively assess drivers of burnout relevant to hospitalists and provide substantive information for hospital medicine groups to develop targeted interventions. Addressing well-being issues requires a focus on systems rather than individuals or teams and improving infrastructure and leadership rather than self-care and personal resilience. This shift requires us to address issues at an organizational level rather than focusing on individual-level solutions.¹³ The instruments reviewed by the authors evaluated a broad range of systems drivers including EHR utilization, leadership engagement, and control over scheduling and workload, among others. If an instrument reveals that control over scheduling is particularly problematic for a team's well-being, hospitalist group leaders can accordingly focus their efforts. Hospitalist leaders may have already identified a specific challenge area and can identify the general category of system drivers they want to assess with the summary table. They can then select the corresponding well-being instrument and do a deeper dive into the more specific drivers assessed in the instrument to see if it captures the desired measurement. Once a hospital medicine group implements a specific strategy, the selected measurement instrument can be utilized again to reassess well-being over time and the impact of the implemented strategy. However, the results of repeat surveys may be difficult to interpret due to other factors that have changed in the same period (for example attrition or the financial status of hospital).

One challenge using systemic driver criteria for evaluating well-being instruments is how expansive "drivers relevant to hospitalists" are. The variability of each instrument reflects this dilemma. Some instruments, such as Mini-Z or HMI, focus on more tangible drivers like control over schedule or workload, with the added benefit of the free text question in the Mini-Z allowing respondents to raise concerns about drivers that are not already listed in the instrument. But there is benefit too, in thinking about drivers which are harder to measure, such as relationships with colleagues and leaders and appreciation. PROMIS would be an appropriate instrument to use in this circumstance. To take it a step further, some instruments try to tackle even more complex and abstract drivers like moral distress—Measure of Moral Distress for Health Care Professionals (MMD-HP)—or professional meaning—Professional Fulfillment Index (PFI). For this latter category of instruments, given the more abstract drivers addressed, one is led back to the question of whether the instrument can lead to change and measure the impact of that change. The OLBI adds a pragmatic component by assessing those at risk for "quiet quitting" via its

disengagement measurements. Each category of drivers, from tangible to abstract, has its value and relevance.

The PFI and Self-Valuation Measures instrument facilitates a more personal assessment of well-being including self-compassion. The CBI takes this a step farther to delineate whether personal vs. work-related issues are contributing most to burnout. The CBI may be most useful in communities under significant stress not necessarily associated with work. The PFI and ProQOL Health instruments also provide participants with feedback and recommendations based on their scores. This can be useful when hospitalist groups want to both measure and provide an intervention for participants.

Three of the instruments, the Maslach Burnout Inventory – Human Services Survey (MBI-HSS), eWBI and PFI, and Self-Valuation Measures, may also allow for comparison among groups and benchmarking of levels of well-being. Benchmarking using well-being measurement instruments poses specific challenges. As already discussed, there is considerable variability in the definition of drivers relevant to hospitalists—both among the instruments and among the multitudes of unique hospital medicine groups. Close review of the instruments also reveals significant ambiguity in the definition of wellness for hospitalists. Column 2 of the summary

table titled “What is being measured/as defined by the instrument” includes a wide range, such as personal health, depersonalization, emotional exhaustion, morale, burnout, moral distress, depression, and personal fulfillment. Given this variety, it would certainly be challenging to benchmark or compare scores across instruments.

An additional challenge of defining and measuring “drivers relevant to hospitalists” is that hospitalists across the country work in varied settings and communities in a wide range of hospitals and work cultures, and drivers will change over time. Drivers most relevant to an academic hospitalist in an urban setting may be very different from drivers most relevant to a community hospitalist working at a small rural hospital. Recent changes in federal funding policies are affecting hospitalists in various ways, especially given the current uncertainty regarding future developments. Hospitalist group leaders will need to tailor the selection of their instrument to the one that best matches their specific setting, their stage of well-being program development, and the driver or drivers they are hoping to target.

Finally, the authors developed a chart with recommendations for the use of individual instruments by leaders of hospital medicine groups. There are limited expert

Table 2 Practical Recommendations for Instrument Use for Hospital Medicine Leadership

Goal	Possible scenarios	Consider using
Understand general level of well-being	Determine baseline prior to starting a well-being program Assess the general impact of a well-being program	Any of the instruments
Use the instrument most commonly cited in well-being studies	I want to look at overall burnout and am willing to pay a bit I want to do well-being scholarly work and want to use what is most commonly used by others in the literature	MBI-HSS
Understanding of various burnout drivers	I want to create interventions that target specific burnout drivers but not sure which yet I want to assess the impact of the above interventions at the systems level	MMD-HP Mini-Z version 2 HMI
Understand if faculty members are considering leaving our group	I want to better explore and quantify concerns related to hospitalist attrition	MMD-HP HMI OLBI
Understand how we do with well-being compared to other groups	I want to benchmark or compare well-being of my group against others	MBI-HSS PFI and Self-Valuation Measures eWBI
Understand how my intervention is impacting a specific burnout driver	I know which driver of burnout I want to target and want to be able to measure it	Use “Categories of driver dimensions assessed” column to identify the driver you wish to study and the corresponding instrument
Understand if major changes in my hospital or community might be impacting well-being	I want to quickly assess the impact of a community disaster or crisis on the well-being of my group I want to quickly assess the impact of a major operational or leadership change on my group	MBI-2 CBI
Use the well-being instrument as both a measurement and a way for participants to think about their own well-being	I want to use the well-being instrument as both a measurement and an intervention for my team	PFI and Self-Valuation Measures ProQOL
Understand if there is a specific issue impacting my team	I suspect one issue may be at play for my group or I am perplexed by our burnout level and want a write-in option for participants	Mini-Z
Understand how several specific variables may be impacting my group’s well-being	I want a customizable instrument that explores groups’ mental social and physical health	PROMIS

recommendations on instrument validity. Therefore, we sought to synthesize the information available through our review and develop consensus statements for best use for the instruments. The goals and scenarios were chosen and edited based on personal experiences as well-being leaders at multiple institutions across the USA and discussion with faculty at national conferences where the authors presented information about the instruments, as well as needs expressed by additional members of the HOMERuN group through surveys.¹⁴ Each author submitted suggestions separately for inclusion in the table, and these were aggregated to develop the first draft. The table was edited in subsequent meetings between the author group until all authors agreed upon the information presented. Examples of recommendations regarding what each instrument is most useful for might include, in the case of a very brief instrument with more general questions, a recommendation to use the instrument for a quick well-being temperature check. In contrast, for a slightly longer instrument with multiple drivers assessed, the authors conclude that it is most beneficial for designing and prioritizing specific interventions. The result, Table 2, is a summary table the authors believe is concise, pragmatic, and most importantly useful for hospital medicine group leadership.

While the authors used a rigorous evaluation process to reduce the introduction of bias, we acknowledge that there are limitations as there is no standard rubric or appraisal method for use with these instruments. To reduce bias, we used a structured study evaluation protocol, comprehensive and systematic literature search, rigorous study selection process, standardized approach to assessment of the quality and relevance of the studies included, and multiple steps where individuals could contribute their ideas to minimize group bias. Authors with knowledge or experience with individual instruments used at their institutions were also open and transparent about their experiences.

CONCLUSION

The goal of this review is to link the rigorous scientific study of well-being to the routine efforts and interventions of hospitalist groups tackling burnout. To accomplish this, the authors conducted a comprehensive review of articles that both involved well-being measurement and were relevant to hospitalists. From this review, the authors culled a list of well-being measurement instruments and then conducted a deep dive into these instruments, filtering them through the lens of utility for hospital medicine. Drawing on their expert opinions, the authors have created an easily accessible framework for selecting the best instrument or instruments for situations

hospital medicine groups may encounter. Hospitalists are problem-solvers and experts in complex systems. Through this work, we hope to make the “science of well-being” accessible to all those hoping to advance well-being in the field.

Future Directions

Upon completion of this review, the authors have two main recommendations moving forward. First, further study and examination are warranted to explore whether the field would benefit from standardization and more ingrained benchmarking of well-being measurement. This work has clarified that this is not a straightforward question given the diversity of hospital medicine groups and the complexity of the work they do.

The second, and perhaps most important recommendation, involves the application of one of hospital medicine’s most important clinical tenets: Think carefully before ordering a test that will not change management. Well-being measurement should be guided by a specific goal. Measuring well-being and drivers of burnout and not acting on these results can breed frustration and dissatisfaction. Measurement of well-being with intention and following with action is essential. Hospital medicine leadership should be proactive in creating a culture of feedback and transparency to improve the yield from these instruments. In addition, similar to reporting data from length of stay and readmission rates, leadership can include well-being survey results in regular report outs to their team.

Through this work systematically reviewing the various instruments available, hospitalist groups will be better equipped not only to measure their group’s well-being but also to create interventions that significantly improve their working environment.

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Author Contribution Ashwini Niranjan-Azadi, Elizabeth Murphy, and Krishna Chokshi assume full responsibility for the integrity of the entire project, from its initial development through the final publication. All authors played a significant role in conceptualizing and designing the project. Ashwini Niranjan-Azadi, Kanapa Kornasawad, and Elizabeth Murphy were responsible for screening the publications for inclusion in the article review. All authors participated in the review of the articles. Ashwini Niranjan-Azadi, Krishna Chokshi, and Elizabeth Murphy prepared the initial manuscript draft, and all

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Declarations

Conflict of interest The authors declare that they do not have a conflict of interest.

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